



Compress attention. Never compress evidence.

A role-aware longitudinal care note where every high-priority statement carries its reason, trust state, and exact immutable source.

PRODUCT THESIS

The glance is an attention budget, not a second timeline.

Three bounded lanes - Act now, Watch, Awaiting - expose the next action while one click restores the full evidence trail.

1.406 ms

MEASURED WARM-PATH P95

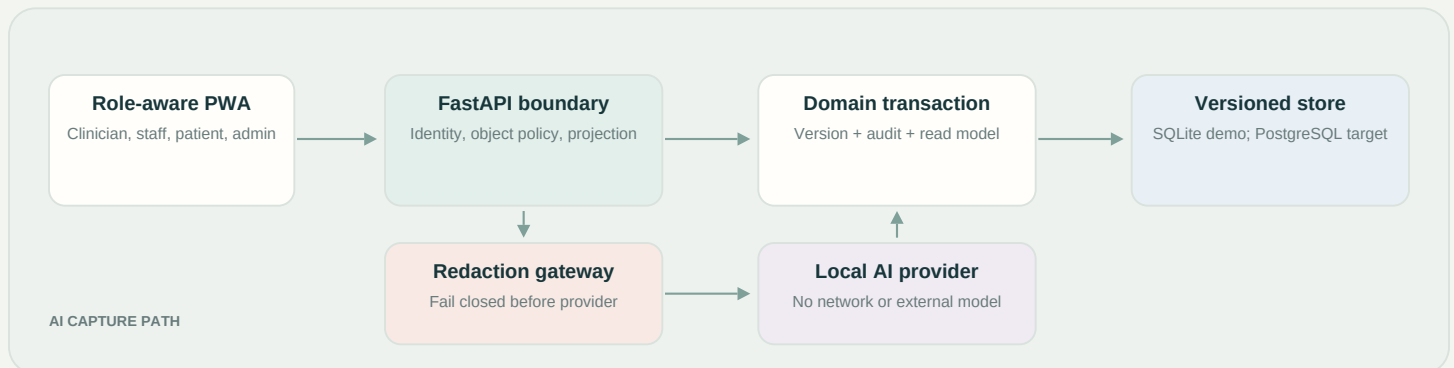
168 tests

BACKEND TESTS; 900 PROPERTY EXAMPLES

100%

BACKEND STATEMENT + BRANCH COVERAGE

ARCHITECTURE



TRUST CONTRACT

- Exact-source highlights bind an entry, immutable version, offsets, quote, source URI, and SHA-256 hash.
- Support is a trust-state policy score (AI 0.65; clinician-confirmed 0.95), not a model-reported or calibrated correctness probability.
- Deterministic allergy and medication rules set a risk floor. Citation-first review extracts exact spans and abstains when evidence is insufficient.
- Patient summaries and instructions require clinician ownership and confirmation. AI drafts remain internal; patient responses use an allow-list projection.
- Revert appends a version. Same-section stale writes return 409; independent role-owned sections do not overwrite.
- The Delta Lens reports temporal change and uncertainty, but refuses a causal claim without an estimand and design.

VERIFICATION CONTRACT

4 x 100%

FRONTEND COVERAGE; 46 TESTS

7 / 7

DESKTOP + MOBILE E2E

2,089 / 2,089

GENERATED MUTANTS KILLED

0 known

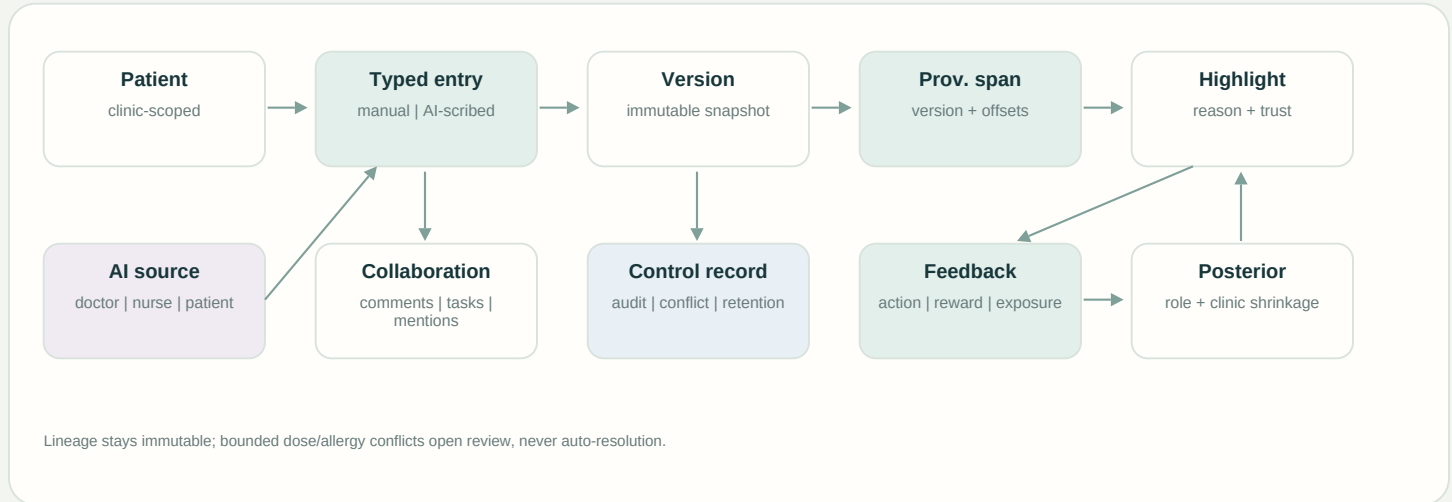
ADVISORIES AT SCAN TIME



A shared narrative with immutable ownership.

Full snapshots favor auditability, deterministic revert, and stable provenance in the prototype. Production can add delta compression behind the same logical model.

COMPREHENSIVE DATA SCHEMA



AUTHORIZATION MATRIX

Object/action	Patient	Staff	Clinician	Admin
Patient-safe summary	Read	Read	Read/write	Read
Staff section	No access	Read/write	Read only	Read only
Clinician section	No access	Read only	Read/write	Read only
Raw AI + comments	No access	Read/review	Read/review	Read
Audit + retention	No access	No access	No access	Operate

ATOMIC INTEGRITY PATH

- 1. Authenticate the server actor; ignore client-supplied role claims.
- 2. Resolve the object inside clinic scope; enforce ownership and visibility.
- 3. Validate expected version; a stale same-section write returns a 409 receipt.
- 4. Append snapshot, metadata-only audit, and glance projection in one transaction.
- 5. Serialize an actor allow-list; provenance rechecks scope, offsets, quote, and hash.

ASSUMPTIONS AND PRODUCTION BOUNDARY

ASSUMPTIONS

Synthetic data, local deterministic provider, and loopback benchmarks.

TRADE-OFF

Full snapshots + SQLite favor auditability and reproduction over storage efficiency and scale.

PRODUCTION GATE - NOT CLAIMED

OIDC/MFA, PostgreSQL RLS, TLS, managed encryption, centralized audit, and independent clinical/security review.



Learning earns influence under safety constraints.

The adaptive system changes ranking, not clinical facts. New policies stay in shadow mode until support, uncertainty, and governance are credible.

TRANSPARENT IMPORTANCE

base = risk + action + safety + recency + pin

adaptive = clip(pooled posterior shift, -0.75, +0.75)

rank = hard safety band, then base + adaptive

- Role and clinic Beta posteriors shrink sparse feedback.
- Critical, allergy, medication, and urgent work are protected.
- Accept, reject, and pin remain one-action human controls.

SHADOW OFF-POLICY EVALUATION

No estimate yet

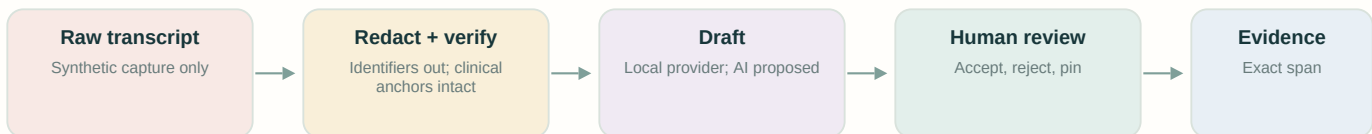
Deterministic exposure (propensity 1.0) reveals surfaced-only feedback bias; the build refuses to manufacture uplift.

REQUIRED DIAGNOSTICS

Exposure bias - propensity overlap - effective sample size - DR uncertainty - assumptions

Ranking proxy only - never a patient-outcome claim

PRIVACY AND RETENTION



Decay applies to recomputable derived caches. Immutable versions, audit metadata, open tasks, conflicts, pins, allergies, medication safety, and active critical evidence remain protected.

MEASURED EVIDENCE AND TRADE-OFFS

600 / 600

WARM READS SUCCEEDED

1.157 ms

LOCAL MEDIAN

1.406 ms

LOCAL P95

P95 <= 300 ms

BRIEF CONSTRAINT PASSED

50 warm-ups; loopback Uvicorn; seeded precomputed projection; no LLM on read path.
168 backend + 46 frontend tests; 7/7 browser; 2,089 / 2,089 mutants; 0 known advisories at scan time.

SELECTED PRIMARY SOURCES - HYPERLINKED

[1] HL7 FHIR R5 Provenance

hl7.org/fhir/provenance

[2] W3C PROV-O

w3.org/TR/prov-o

[3] OWASP API1:2023 BOLA

owasp.org/API-Security/2023/API1

[4] PostgreSQL Row Security

postgresql.org/docs/current/ddl-rowsecurity

[5] NIST AI 600-1 Generative AI Profile

doi.org/10.6028/NIST.AI.600-1

[6] PDPC Healthcare Sector Guidelines

pdpc.gov.sg/healthcare-sector

[7] Dai et al., AI-scribe safety signals, 2025

arxiv.org/abs/2512.04118

[8] Brunner et al., JAMA ambient scribe, 2026

doi.org/10.1001/jamanetworkopen.2025.52870

[9] Mandyam et al., CANDOR (CHIL 2026)

proceedings.mlr.press/v333/mandyam26a

[10] W3C WCAG 2.2

w3.org/TR/WCAG22

[11] Owens et al., patient portal AI, 2026

jamanetwork.com/journals/jamanetworkopen/2851278

[12] Cost-aware selective deferral, 2026

nature.com/articles/s41598-026-40637-w